Enhanced Preventive Procedures

Should All Child Care Solutions Inc. be required to continue to operate, there are additional enhanced preventive procedures which will be put in place to further reduce the risk of infection.

The enhanced preventive procedures which the Director, home visitor and home provider will follow is dependent on the following guiding principles:

- Exclusion
- Open Communication between Providers and Families
- Use of Personal Protective Equipment
- Changes to Daily Routines to Minimize the Spread of Infection
    - Monitoring, Social Distancing and Cleaning and Disinfection Procedures
- Agency team Structure
- Home Visitor & Director's Role

Exclusion

No.	Scenario	Children & Provider
1.	Individuals returning form travel.	Must self-quarantine for 14 days and must be 72 hours symptom free without medication before being permitted to return to the provider's home.
2.	Individuals living in the same household as persons returning from travel.	Must self-quarantine for 14 days and must be 72 hours symptom free without medication before being permitted be around children in the home (must self isolate)
3.	Individuals exhibiting symptoms of Covid-19 prior to entering the provider's home: • Shortness of breath. • Dry cough. • Fever.	Must self-quarantine and get tested for Covid-19. If the individual **cannot** be tested for any reason, they must self-quarantine for 14 days and must be 72 hours symptom free without medication before being permitted to return to the provider's home. If an individual **is tested** for Covid-19, the individual must wait for their test results to come back and self-quarantine in the meantime. If the individuals' test results are **negative**, they can return to the provider's home. If the individuals' test results are **positive**, refer to *Scenario 9* immediately, **AND** – Notify local Childcare Licensing agency, **AND** – Follow the regular *Ontario Serious Occurrence (SO)* **OR** *Unusual Incident* reporting requirements, Including submitting a SO report (Ontario) in CCLS and posting the SO notification Form.
4.	Individuals who show no symptoms but who are living in the same household as persons who have **not** been tested but exhibit symptoms of Covid-19.	Must self-quarantine for 14 days and be 72 hours symptom free without medication. (Must Self Isolate) Care may be temporarily suspended in provider's home. **Note:** If individual starts exhibiting symptoms of COVID-19, please refer to *Scenario 3* and the self-quarantine period resets.
5.	Individuals who are showing no symptoms but who are living in the same household as persons with a **positive** Covid-19 test.	Must self-quarantine for 14 days and be 72 hours symptom free without medication. Care may temporarily be suspended in provider's home. **Note:** If individual starts exhibiting symptoms of COVID-19, please refer to *Scenario 3* and the self-quarantine period resets.
6.	Individuals who are showing **no** symptoms and who are living in the same household as persons with a **negative** Covid-19 test result.	Individuals can resume and continue care in provider's home.

7.	Individuals with other illnesses, e.g., *cold, runny nose, vomiting, diarrhea*.	Individuals must be excluded for 72 hours and be symptom free, without the use of medication.
8.	If I encounter an individual who I find out later tested **positive** for Covid-19.	Must self-quarantine for 14 days and be 72 hours symptom free without medication. **Note:** If individual starts exhibiting symptoms of COVID-19, please refer to *Scenario 3* and the self-quarantine period resets.
9.	Individuals exhibiting symptoms of Covid-19 while at the provider's home: • Shortness of breath. • Dry cough. • Fever.	The individual must be excluded from the facility immediately. If the individual is a child the child must be immediately isolated from other children and provider, and parents must be called to come and pick up the child immediately. (Refer to *Additional Protocols When an Individual Demonstrates Symptoms of Illness or Becomes Sick at the provider's home*). Immediately call the Public Health Department. and Report Serious Occurence Take appropriate instructions from Public Health for next steps, including informing other families with children in same provider's home. Please wait until Public Health **authorizes** you to share the information with parents. Follow the regular *Ontario Serious Occurrence* (SO) **OR** Unusual Incident reporting requirements, Including submitting a SO report (Ontario) in CCLS and posting the SO notification Form. Report the situation to the Agency Director. Contact information for the Agency Director can be found on the *Emergency Information Posting (EIP)* located on the information board. The Agency Director will report to "M-edu" and maintain communications throughout the process.
10.	If you become aware that a provider or child in the home tested positive for COVID-19.	Refer to *Scenario 9*.

Additional Protocols When an Individual Demonstrates Symptoms of Illness or Becomes Sick at the Provider's Home

1. If a separate room is not available, the sick person should be kept at a minimum of 2 meters from others.
2. The sick person should be provided with tissues and reminded of hand hygiene, respiratory etiquette, and proper disposal of tissues.
3. If the sick person is a child, provider should remain with the child until a parent arrives. If tolerated and above the age of 2, the child should wear a surgical mask. The childcare provider should wear a surgical mask and eye protection while waiting. The childcare provider should also avoid contact with the child's respiratory secretions. Must wear gloves if child needs attending to.
4. All items used by the sick person should be cleaned and disinfected. All items that cannot be cleaned (paper, books, cardboard puzzles) should be removed and stored in a sealed container for a minimum of 7 days.

Maintain Open Communications – Providers & Parents

Communication is Essential!

1. Maintain open communication with your **agency and parents**. Share all updates including status and emergency procedures.
2. Providers at home, keep informed via email and telephone calls.
3. Post a description of the symptoms associated with COVID-19 at the entrance and inside the home and ask parents to seek professional medical assistance if they are concerned about their child:
    a) Canada: https://www.canada.ca/en/public-health/services/diseases/2019-novel-coronavirus-infection/symptoms.html#s.
    b) US: https://www.cdc.gov/coronavirus/2019-ncov/prepare/children.html.
4. Emergency contact: refer to the *Emergency Information Posting (EIP)*.
5. Inform parents of all changes to procedures so they are aware and are following them.

Use of Personal Protective Equipment (PPE) for providers

It is important for providers to use PPEs as per recommendations of Local Public Health. Refer to the PPE grid on the use of PPEs in the childcare setting.

Daily Routines and Procedures for Providers & Volunteers

Practice social distancing and include cleaning and disinfection as a very important part of your daily routines.

Opening

1. Provider must always wear a protective face mask when working at the home, this is for their own protection and the protection of the children and parents. The following resources will help providers to make their own face mask if they are unable to obtain a proper face mask:

    a) https://www.youtube.com/watch?v=tPx1yqvJgf4&feature=youtu.be.

    b) https://www.cdc.gov/coronavirus/2019-ncov/prevent-getting-sick/diy-cloth-face-coverings.html.

2. Ensure that the playroom (including play materials) has been cleaned, disinfected and ready to use.

Pick-Up and Drop-Off

1. Avoid all physical contact with parents – No handshakes, hugs etc.
2. Monitor children's temperature:
    a) **On arrival** at the home, **and any time** a child shows symptoms of illness.
    b) The Home Visitor/Director will print the attached Daily Health Check Form to monitor the temperature of all children at three evenly spaced times throughout the day.
    c) **NOTE:** The Provider must use the Record of Illness Form – Individual to record information (symptoms, age, room, etc.) concerning an ill child.
3. Ask parents to drop-off and pick-up their children at staggered intervals (to avoid interaction).
4. Parents should drop-off children at the main entrance and have the provider bring the child into their home. Parents may wait outside the main entrance at pick-up time.
5. Designate an area near the main entrance to conduct screening. The area should allow for physical distancing (minimum 2 meters) between provider and the person being screened. If physical distancing cannot be maintained, providers conducting screening should wear appropriate personal protective equipment, such as mask and gloves. *Alcohol based hand rub (ABHR)* containing at least 60-90% alcohol content should be available at the screening station for hand hygiene.
6. Actively screen parents and children, prior to entry/drop-off for respiratory symptoms. A central record of monitoring should be kept. Examples of screening questions include, but not limited to the following:

    a) Do you or the child or any member of your household have any of the following symptoms:

- Fever
- New or existing cough
- Difficulty breathing
- Sore throat
- Vomiting and/or diarrhea?

b) Have you or the child had close contact (within 2 meters) in the last 14 days with a person with acute respiratory illness?

c) Have you or the child had close contact (within 2 meters) in the last 14 days with someone that is a confirmed/ probable case of COVID-19 (including those who are waiting for COVID testing result)?

d) Have you or the child travelled outside of Canada, including the United States, within the last 14 days?

7. Anyone who answers **YES** to any of the question should not be permitted to enter the provider's home.
8. Make ABHR available at the screening stations for people who have answered **NO** to all questions for use prior to entry into the home. When ABHR is used on children they must be supervised. To ensure proper use, follow the manufacturer's directions.
9. Thermometers must be covered with single-use protective covers (which is disposed after each use) or cleaned and disinfected with an outbreak level disinfectant before reuse.
10. Post signage in visible areas which clearly explains the screening process and the rules and conditions for entry.
11. Handwashing for children will be done **immediately upon arrival**.

Coat Room Spaces

1. Use alternative hooks (so that children's clothes and objects do not touch each other).

Activities

1. It is ok to explain the current situation to children, without unduly worrying them, in a language understandable to them: https://www.huffingtonpost.ca/entry/coronavirus-kids-parents_ca_5e31f632c5b6b7612c092544.
2. Keep at least a 2-meter distance between children when conducting activities.
3. Reduce the number children gathering in any learning area.
4. Move children amongst learning areas while the other areas are disinfected.
5. Avoid activities that require children to share toys or touch each other (holding hands etc.). If playing board games, let only one child move the pieces.
6. Limit group sizes (E.g. washroom – do not line up).

7. Suspend all sensory play (E.g. water table play, sensory food play).
8. Handwashing will be made a new routine and fun-activity which will be conducted at every hour/two-hour interval, or more frequently.

Interactions with Infants and Toddlers

1. Provider must continue to supervise and hold bottles for infants not yet able to hold their own bottle to reduce the risk of choking.
2. When holding infants and toddlers use blankets or cloths over clothing and change the blankets or cloths between children.
3. Consider removing cribs or placing infants in every other crib and mark the cribs that should not be used to support physical distancing.
4. Recognizing that physical distancing is difficult with small children and infants, suggestions to support physical distancing include:
    a) Planning activities that do not involve shared objects or toys; and,
    b) When possible, moving activities outside to allow for more space.
5. Children must not share food, feeding utensils, soothers, bottles, sippy cups, etc. Mouthed toys must be removed immediately for cleaning and disinfecting and must not be shared with other children.
6. Label these items with the child's name to discourage accidental sharing.

Ratios

1. Follow the ratios rules and room capacity rules as mandated by Childcare Licensing.

Snack and Mealtimes

1. Follow all existing rules for cleaning and disinfection and hand hygiene before, during and after meals.
2. Keep one chair distance apart at snack and mealtimes when children are eating.
3. Providers must wear gloves when serving food.
4. Use disposable cutlery where possible.
5. There should be no sharing of utensils.
6. All food preparation and portioning, including snacks, must be done in the kitchen.
7. There is no self-serve or sharing of food. Provider serves all food to the children.
8. Limit the number of personal water bottles. Use cups instead and send them to be washed immediately. Use paper cups if available.
9. Food remains should not be reused but thrown in the garbage right way.
10. Provider must return dirty dishes and cutlery to the sink for disinfecting.

12. If available, all dishes and cutlery must be washed in a dishwasher.
13. There should be no food provided by the family/outside of the regular meal provision of the program (except where required and special precautions for handling and serving the food must be put in place).
14. Children should neither prepare nor provide food that will be shared with others.

Nap & Rest Time

1. If possible, maintain a distance of one cot apart spacing when children sleep.
2. If possible, in the infant sleep area, maintain a distance with alternative cribs spacing when infants sleep.
3. Each infant's bedding must be used for him/her only. All bedding must be laundered when wet or soiled, additionally: (If Provider launders beddings)
    a) Wet or soiled bedding must be placed in a suitable container until it can be laundered.
    b) Open laundry machine door first before touching dirty clothes to transfer.
    c) Cots, cribs, and mattresses must be wiped daily with an appropriate disinfectant, and when wet or soiled.
4. Wash hands after clearing up the sleep area before returning to the children.

Washroom & Diaper Changing Routines

1. Ensure toilet seats and faucets are cleaned and disinfected after each use.
2. Ensure that diaper changing routines are followed thoroughly as per usual.

Outdoor Time

1. Stagger outdoor time to small groups of children.
2. Avoid usage of balls and other toys that require frequent sharing.
3. Disinfect riding toys between uses.
4. Where the outdoor play area is large enough to accommodate multiple kids, providers may divide the space with physical markers to ensure children remain separated by at least 2 metres.
5. If play structures are to be used by more than one child, the structures can only be used by one child at a time and should be cleaned and disinfected before and after each use by each child.
6. Where there are challenges securing outdoor play space, we can have alternate outdoor arrangements (e.g. community walk). Follow physical distancing practices when possible.

Closing

1. Ensure that all areas (including play material areas) that you will be using the next day are cleaned, disinfected and ready to use.

Provision of Special Needs Resources (SNR) Services

1. Childcare Licensing recognizes that children with special needs and their families continue to require additional supports and services in childcare settings.
2. The provision of in-person special needs services in childcare settings should continue where appropriate. Should questions arise in respect of which service providers are permitted to enter the premises, please consult with your local public health unit. Please work with special needs service providers to explore alternative modes of service delivery where in-person delivery is not possible.
3. Maximum capacity rules do not apply to SNR staff (consultants and enhanced staff) on site (i.e., if they are not counted towards staff to child ratios, they are not included in the maximum capacity rules).
4. Where SNR services are provided through external service providers, licensees and home childcare providers should inform all families of this fact, and record attendance for contact tracing purposes.
5. All SNR staff must be screened before entering the childcare setting, as per the protocol in the screening section above.

Cleaning and Disinfection

Increase Frequency of Cleaning and Disinfecting

1. Twice daily - High-touch areas – doorknobs, handles, phones, tablets (iPads), light switches.
2. Daily - Counters, chairs, tables, toilets, and sinks (after each child has sat/used that area).
3. Put away toy for cleaning and disinfecting as soon as it is mouthed.
4. Floors in the home with children are cleaned twice a day. And once a day for floors without children (e.g. kitchen, hallways).
5. bathrooms, and common areas disinfected and cleaned frequently.

Home Cleaning

1. Follow Public Health Requirements OR
2. Unless specified here, double cleaning procedures based on existing All Child Care Solutions Inc. requirements.

Toys and learning Material

1. Toys and learning material must be cleaned and disinfected at least once per day.
2. Limit the number of learning materials to use. E.g. in Dramatic area reduce the number of cups, or dolls.
3. Remove materials that are difficult to clean – natural materials, dramatic play clothes, stuffed toys.
4. Replace with toys that you can possibly disinfect in a dishwasher.
5. Quarantine toys that you are unable to clean (put it in a plastic bag, label).
6. Toys and equipment should be cleaned and disinfected at a minimum daily!
7. Have designated toys and equipment (e.g., balls, loose equipment) for each child. Where toys and equipment are shared, they should be cleaned and disinfected prior to being shared.
8. If sensory materials (e.g., playdough, water, sand, etc.) are offered, they should be provided for single use (i.e. available to the ONE child for the day) and labelled with child's name.

Other Areas & Items

1. Coat Room – clean and disinfect at the end of each day.
2. COTS – clean and disinfect at the end of each day.
3. SHEETS, covers, blankets – Wash and dry as required.
4. Where applicable, provide a cup of 'Clean' pens for parents to use when signing the *Sign-in Sheet* upon dropping off their child. Place the used pens in a cup marked 'Used'. 'Used' pens must be disinfected before placing back in the 'Clean' cup. Alternatively, ask parents to use their personal pen.

Use the Cleaning Schedule Form to track your cleaning routines so not to miss anything. As much as possible, cleaning and disinfecting must be carried out with children not in the room, or in a separate room.

Personal Hygiene

1. Wash hands, every hour, after wiping noses, and after using the toilet (or use hand sanitizer).
2. Make sure to cough and sneeze into your elbow.
3. Follow the All Child Care Solutions Inc. Handwashing Posting in the absence of Public Health instructions.

High-Level Disinfectant (Preparation)

Providers must always use high-level outbreak disinfectant when disinfecting areas and items in the Providers Home.

Method 1

To prepare a high-level disinfectant with bleach, follow the method below. We prepare a disinfectant liquid with 5000 PPM of chlorine.

Concentration	**5,000 PPM**
Used for	High-level disinfectant used for items and surfaces contaminated with blood, body fluids and feces / vomiting
Preparation method	**Mix 7.5 tablespoons of bleach with 1 liter of water. Prepare fresh mix every morning.**
Contact time	**10 minutes**

Method 2

Obtain Zochlor tablets: https://b2b.sanimarc.com/product.aspx?prodid=7253&cat=WW

Preparation method: Use 5 tablets / 1000ml of water to produce 5000 PPM strength disinfecting solution.

Contact time 2 minutes.



Home Visitors & Directors

1. Have only the necessary number of employees on site to meet the needs of the families.
2. Have the same set of providers (including supply / replacement) work with the same group of children as much as possible. The more you reduce additional contact with different people the less the spread of infection.
3. When planning provider rotations:
   - **a)** Avoid providers working across several home locations
   - **b)** Have one provider work the full day schedule.
   - **c)** Make sure the morning home visitor does not meet the afternoon homevisitor. (where applicable)
   - **d)** Cleaning is every Home Providers' responsibility.

Monitoring and Communicating

1. Ensure providers are carrying out their enhanced routines regularly and correctly.
2. Ensure you can communicate with local Public Health in a timely manner for any issues that emerge.
3. Do not hesitate to reach out to the local Public Health Department for directions on situations.

Preventive Procedures for Home Visitors and Directors

1. Director/Home Visitor must be in contact as much as possible to ensure compliance with the instructions to parents, children, and prioviders.
2. Stay in the office as much as possible and make sure to follow up with the team working with children while respecting the principle of social distancing.
3. Be open without giving into panic, remain flexible and benevolent with your work teams.

Preventive Procedures for Suppliers

1. Suppliers, including the catering company, and visitors are not authorized to enter the provider's home.
2. The supplies should be left at the entrance in a box identified for this purpose or in front of the rear door.
3. Inform suppliers that you must apply strict measures to limit the spread of the virus.
4. If you must buy food at the grocery store, encourage online shopping and delivery. When you receive the delivery, limit handling or movement in the home. Clean the surfaces that have been in contact with items.

Personal Protective Equipment (PPE)

PPE Item	Child Arrival	Child Departure	Children less than 2y old. (ALL THE TIME)	Any other time of the day & When with children 2y+	Ill child Or showing symptoms of illness	Diapering / Toileting Routine	Making Disinfectant	Kitchen Routine (washing dishes, cleaning) or Cleaning / Sanitizing Routine (mopping, sweeping, garbage removal)	Laundry Routine	@ End of Day
Masks	✓	✓	✓	✓(SR)	✓	✓(SR)	✓(SR)	✓(SR)	✓(SR)	Garbage
Gloves	✓	✓	✓		✓	✓	✓	✓	✓	Garbage
Goggles	✓	✓			✓		✓			Disinfect*

NOTE 1: A cell with ✓ indicates mandatory. If cells are blank, the use of the PPE item is at the discretion of the home provider.

NOTE 2: A cell with ✓(SR) indicates strongly recommended.

NOTE 3: * Disinfect only if the item is reusable.

	Date Completed	Signature
I, …… , working in the position of ……., have read and understood **the COVID-19 Preventive Procedures in place, and the instructions I need to follow to carry out my duties.** I declare that I will follow this procedure.		
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PERSONAL PROTECTIVE EQUIPMENT

PUT ON Personal Protective Equipment in this order:

1. 
PERFORM HAND HYGIENE

2. 
GOWN

3. 
MASK

4. 
GOGGLES

5. 
GLOVES

TAKE OFF Personal Protective Equipment in this order:

1. 
GLOVES

2. 
GOWN

3. 
PERFORM HAND HYGIENE

4. 
GOGGLES

5. 
MASK

6. 
PERFORM HAND HYGIENE

EMPLOYEE HANDWASHING

EMPLOYEES MUST WASH HANDS BEFORE RETURNING TO WORK.

1.



WET HANDS

Wet hands with running water.
At least 100°F / 38°C

2.



APPLY SOAP

3.



LATHER AND SCRUB

Lather and scrub - 20 seconds
Scrub lathered fingers, fingertips, between fingers, hands and arms.

4.



RINSE

Rinse under clean running water - 10 seconds

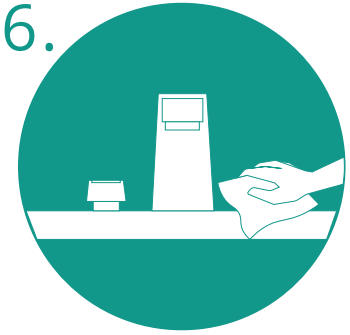
5.



DRY HANDS

Dry your hands thoroughly with a single use towel.

6.



TURN OFF FAUCET

Use towel to turn off faucet.